THE WHITE ARMY
#ComprehensiveClinicalCases

1.ENT-EAR
Patient particulars
Name
Age
Gender
Address
Education.
Occupation
Date of admission
Date of examination

Chief presenting/complaints
Hearing loss.
Tinnitus.
Dizziness or vertigo.
Ear discharge.
Earache.
Itching in the ear.
Deformity of the pinna.
Swelling around the ear

History of presenting complaints/illness:
Hearing loss:
Onset
Duration
Progression
Unilateral/Bilateral
Trauma
Associated symptoms

Earache:
Onset
Duration
Progression
Nature of pain
Radiation
Associated throat pain, symptoms of upper respiratory tract infection ,discharge, hearing loss

Discharge:
Onset
Duration
Progression
Colour
Blood tinged/not
Foul smelling
Amount
Aggravating factor
Relieving factor
Associated symptoms
Symptoms of Upper respiratory tract infection

Tinnitus:
Onset
Duration
Progression
Aggravating factor
Reliving factors
Associated symptoms

Also ask,
Fever
Itching of the Ear
Throat irritation
Swelling of the Ear

Deformity
Throat pain
Dysphagia
Odynophagia
Change /Hoarseness of voice
Palpable swelling any Where in the neck
Nasal obstruction
Running nose
Discharge from nose
Cough
Watering of eyes.

Treatment history
History of allergy to food and any drug
Past history
Family history
Personal history
Menstrual history (In females)
Marital and obstetric history (In females)
Environmental history
Socio-economic history
Summary after history

General physical examination

Patient is conscious /not ,cooperative/not.
Orientation to time ,place ,person.
Built
Nourishment.

Vitals:
Pulse :
Blood pressure.
Respiration

Temperature.

Pallor
Icterus
Cyanosis
Clubbing
Edema
Lymphadenopathy

Height
Weight
BMI.

Examination of the Ear.

A.Physical examination.
1.Pinna and the surrounding area.
-Inspection

Both of its surfaces
The lateral and the medial
Look for size
Shape
Position
Redness
Swelling
Vesicles in concha and retroauricular groove
Scars
Ulceration or neoplasm.

-Inspection of area above, in front, below and behind the pinna:
swelling
sinus
fistula
scar

-Palpation of pinna
Raised temperature
Thickness of tissues
Fluctuation
Tenderness.
Movement of pinna

2. Examination of external auditory canal:

*Examination without a speculum.
Size of meatus
Contents of lumen
Swelling of meatal wall

*Examination with a speculum.
Wax
Debris
Discharge
Polyp
Granulations
Exostosis
Benign or malignant neoplasm
Sagging of postero-superior area

3. Examination of tympanic membrane.
Colour.
Position.
Surface of tympanic membrane.
Mobility.

4.Examination of middle ear. Normally, middle ear cannot be examined directly. When tympanic membrane is semitransparent, some structures can be seen through it. In the presence of a perforation, it is possible to know the condition of middle ear mucosa and any in-growth of squamous epithelium from the edges of the

perforation.

5. Examination of mastoid.
Swelling
Obliteration of retroauricular groove
Fistula
Scar

*Palpation of the surface of mastoid
Mastoid tenderness
Over the antrum
Over the tip
Over the part between the mastoid tip and mastoid antrum.

6. Examination of eustachian tube.
*Tympanic orifice of eustachian tube can be seen in the anterior part of middle ear if there is perforation of tympanic membrane.
*Pharyngeal opening of tube can be seen by posterior rhinoscopy.
*Function of tube can be tested by Valsalva manoeuvre.

7. Examination of facial nerve.

B.Functional examination.

1. Auditory function
Voice test
Tuning fork tests
•Rinne test
•Weber test
•Schwabach test
•Absolute bone conduction test.

2. Vestibular function
Spontaneous nystagmus
Fistula test

Positional tests

Examination of nose
Examination of Throat

Systemic examination.
Central nervous system
Cardiovascular system
Respiratory system
Abdomen

Summary
Provisional diagnosis
Differential diagnosis
Management-Investigation and Treatment



2.ENT-Nose

Patient particulars

Name
Age
Gender
Address
Education
Occupation
Date of admission
Date of examination

Chief/presenting complaints

Nasal obstruction.
Nasal discharge.
Postnasal drip.

Sneezing.
Epistaxis.
Headache or facial pain.
Swelling or deformity.
Disturbances of smell.
Snoring.
Change in voice (hyper- or hyponasality).

History of presenting complaints/illness.

Nasal obstruction:
Onset
Duration
Progression
Aggravating factor
Relieving factor
Associated symptoms
Diurnal /seasonal variation

Nasal discharge:
Onset
Duration
Progression
Amount
Colour
Diurnal variation
Seasonal variation.
Blood tinged
Foul smelling
Aggravating factor
Relieving factor
Associated symptoms

Postnasal drip:
Onset

Duration
Associated symptoms

Sneezing:
Onset
Duration
Progression.
How often
Number of episodes
Associated symptoms
Diurnal variation

Epistaxis:
Onset
Duration
Associated symptoms
Hypertension
Trauma
Systemic illness

Also Ask,
Headache or facial pain.
Swelling or deformity of the nose
Disturbances of smell.
Snoring.
Change in voice
Dysphagia
Congenital anomalies
Throat pain
Throat irritation
Cough
Expectoration
Earache
Ear discharge
Hearing loss

Tinnitus

Treatment history
History of allergy to food and any drug
Past history.
Family history
Personal history
Menstrual history (In females
Marital and obstetric history (In females)
Environmental history

Socio-economic history
Summary after history

General physical examination.

Patient is conscious /not ,cooperative/not.
Orientation to time ,place ,person.
Built
Nourishment.

Vitals:
Pulse :
Blood pressure.
Respiration
Temperature.

Pallor
Icterus
Cyanosis
Clubbing
Edema
Lymphadenopathy
Height

Weight
BMI.

Examination of the Nose.

1.External nose:
*Inspection:
1)Skin :
Scars
Sinus
Swelling or a neoplasm
2)Osteocartilaginous framework :
Deformity
Deviated or twisted nose
Hump
Depressed bridge,
Bifid or pointed tip,
Destruction of nose

*Palpation of nose :
Raised temperature
Fixity of skin
Thickening of soft tissues,
Tenderness
Fluctuation or crepitation.

2.Vestibule:
See For,
Furuncle
Fissure
Crusting,
Dislocated caudal end of the septum
Tumours

3.Anterior Rhinoscopy.
Nasal passage
Septum
Floor of the nose
Roof
Lateral wall

4.Posterior Rhinoscopy.

5. Functional examination of Nose
Patency of nose
Spatula test
Cotton wool test.
Sense of smell

Examination of Paranasal Sinuses.

1.Maxillary sinus.

*The soft tissues of cheek, lip, lower eye lid and the molar region.
*The orbit and its contents, and the vision
*The vestibule of mouth by everting the lip
*Upper alveolus, teeth and palate
*The nose by anterior and posterior rhinoscopy *Tenderness by pressure over the canine fossa

2.Frontal sinus.
External examination
Examination of nose

3.Ethmoid Sinus
External examination
Examination of nose.

4.Sphenoid sinus.
Anterior Rhinoscopy

Posterior Rhinoscopy.

Examination of Throat

Examination of ear.

Systemic examination

Central nervous system
Respiratory system
Abdomen
Cardiovascular system



Summary
Provisional diagnosis
Differential diagnosis
Management-Investigation and Treatment.

3.ENT-Throat

Patient particulars

Name
Age
Gender
Address
Education
Occupation
Date of admission
Date of examination

Chief presenting/complaints.

Sore throat
Odynophagia
Dysphagia
Change in voice
Earache
Snoring
Halitosis
Hearing loss
Abnormal appearance of Throat

History of present illness.

Sore throat:
Onset
Duration
Progression
Associated symptoms

Odynophagia:
Onset

Duration
Progression
Aggravating factor
Relieving factor
Associated symptoms

Change in voice:
Onset
Duration
Associated symptoms

Earache :
Onset
Duration
Progression
Nature of pain
Aggravating factor
Relieving factor
Associated symptoms

Also Ask,
Pain
Disturbance in salvation
Disturbance of taste
Trismus
Lesion in oral cavity.
Halitosis
Hearing loss
Abnormal appearance of Throat
Ear Discharge
Congenital anomalies
Nasal obstruction
Nasal discharge
Running nose
Palpable swelling any Where around the neck

Fever
Symptoms of Upper Respiratory system infection.

Treatment history
History of allergy to food and drug.
Past history
Family history
Personal history
Menstrual history (In females)
Marital and obstetric history(In females)
Environmental history
Socio-economic history
Summary after history

General physical examination.

Patient is conscious /not ,cooperative/not.
Orientation to time ,place ,person.
Built
Nourishment.

Vitals:
Pulse :
Blood pressure.
Respiration
Temperature.

Pallor
Icterus
Cyanosis
Clubbing
Edema
Lymphadenopathy

Height
Weight
BMI.

Examination of oral cavity.

1.Lips
Swellings
Vesicles
Ulcers
Crusts
Scars
Unilateral or bilateral clefts.

2.Buccal mucosa
Change in colour
Change in surface appearance

3.Gums and Teeth
Red and swollen gums
Ulceration
Gums covered with a membrane.
Hyperplasia.
Growths.
Loose teeth.
Carious infected tooth or teeth
Malocclusion.
Fractures of mandible or of teeth maxilla Abnormalities of Temporomandibular joint.

4.Hard palate
Cleft palate
Oro-nasal fistula
High-arched palate
Bulge
Bony growth in midline

Mass or ulcer

5.Tongue
Look for ,
Large size
Ulcer
Fissure
Inability to protrude
Bald tongue
Deviation on protrusion
White thick patch on the tongue
Growth on tongue

6.Floor of the mouth.
Scar
Ulcer
Swelling

7.Rertromolar trigone

Examination of oropharynx

1.Tonsils and pillars
Tonsils
*Presence.
*Size.
*Symmetry
*Crypts.
*White or yellow spots .
*Membrane
*Ulcer
*Mass
*Bulge
2.Pillars.
*Congestion
*Ulcer

3.Soft palate

4.Posterior pharyngeal wall

5.Base of the tongue and Valleculae.

Examination of nasopharynx.

1.Anterior Rhinoscopy
2.Posterior Rhinoscopy
3.Cranial nerve examination
4.Lymph node examination.

Examination of the larynx.

1.External Examination of the Larynx.
Redness of skin
Bulge or swelling
Widening of larynx
Surgical emphysema
Change in contour of laryngeal structures
Movements of larynx.

2.Indirect laryngoscopy

3.Assessment of Voice.

4.Assesment of Cervical lymph nodes.

Examination of Lymph nodes of the Neck

Examination of Ear

Examination of Nose

Systemic examination

Central nervous system
Cardiovascular system
Respiratory system
Abdomen

Summary

Differential Diagnosis

Provisional Diagnosis

Management-Investigation and Treatment

